جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : هدى جمال الدين فؤاد احمد
المحافظة : القاهره **الحي / المدينة :** القاهره الجديده **العنوان :** التجمع الخامس
رقم البطاقة : 26602052102128 **رقم التليفون :** 01221112015 **تاريخ تسجيل الإستمارة :** 10-10-2026
العمر(تاريخ الميلاد) : 05-02-1966 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالغشاء البللوري
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالغشاء البللوري
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [6399] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: هدى جمال الدين فؤاد احمد
National ID: 26602052102128

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of pleural mesothelioma for Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS

افنيتور



National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

04/16/2026

MDT FORM

Name:	Hoda حدل فوزي احمد	MRN:	10107
Age:	60 Y	ID:	26602052102128
Menopause Status:	post	Phone:	01122261635
NRS SCORE:		Marital Status:	married
BMI:		Family History:	-ve
CO Morbidity:	DM	Reason Of MDT:	

Clinical Data:

BX:

ER 5/8 **PR** 3/8 **HER** 0 **KI67** 0%

Sonomammography:

Mets Workup:

History:

Female pt 60 yrs old , DM , Lt breast cancer 2003 , underwent BCS , received adj. CTH and PORT , Tamoxifen 5 yrs , then kept under FU till 2010 with local recurrence , received AI till 2016 when became PD , underwent MRM , received Faslodex till 2023 , kept under FU till 2024 when developed ascites (peritoneal mets) and received Kisqali+AI till 11/2025 PD , received Taxol/Carb 9 weeks with partial

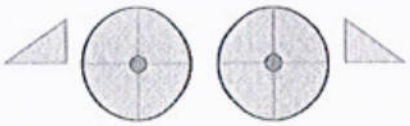
Recommendations:

For MDT , referred for fulvestrant+everolimus after [illegible]

MDT Outcome & Plan

Review Pathology at Post MDT Clinic: if Luminal to start Aromasine -Affinitor.

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ سمر حسن					

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: هدى جمال الدين فواد احمد **MR. :** 10107	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		**رقم طبي:**	

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : | **تاريخ وساعة الخروج :**

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 60-year-old individual with a history of diabetes mellitus and left breast cancer diagnosed in 2003. Initial management included breast-conserving surgery, adjuvant chemotherapy and radiotherapy, followed by five years of tamoxifen. Surveillance continued until 2010 when a local recurrence was identified; an aromatase inhibitor was administered until 2016, at which time disease progression prompted a modified radical mastectomy. Subsequent therapy with Faslodex was continued until 2023, after which the patient was followed until 2024 when ascites due to peritoneal metastases developed. Treatment with Kisqali plus an aromatase inhibitor was given until November 2025, when progression was noted. The patient then received Taxol/Carboplatin for nine weeks, achieving a partial response; the last dose was on 17 March 2026 but was complicated by a severe hypersensitivity reaction. A PET-CT performed on 4 April 2026 demonstrated metabolic and morphologic regression of the nodular peritoneal lesions without new lesions. Multidisciplinary discussion recommended initiation of fulvestrant plus everolimus after pathology revision.

Pathology revision showed estrogen receptor 8/8, progesterone receptor 8/8, HER2 score 0, and Ki-67 20 %. The plan was to start aromasin plus affinitor; the patient independently initiated Faslodex and was referred to the Ministry of Health for coverage of affinitor.

The patient was prescribed aromasin 1 mg daily and affinitor 10 mg daily, with the next clinic visit scheduled for 12 July 2026.

At the follow-up visit, the patient continued aromasin 25 mg and affinitor 10 mg daily. Random blood glucose was 220 mg/dL and HbA1c was 9.3 % (showing improvement). The patient was advised to follow up with endocrinology and to repeat laboratory studies as needed; the regimen was accepted.

The most recent visit documented continuation of aromasin and affinitor 10 mg daily, with laboratory monitoring deemed appropriate and the patient cleared for the next cycle of therapy.

تحريرا في ,

التاريخ : 20-09-2026 **الوقت :**

الطبيب المعالج : خالد عبد العزيز محمد كمال

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / هدى جمال الدين فؤاد	Request No.	: 13006052
Patient No.	: 26602052102128	Request Date	: 22/04/2026
Path No.	: SR-496/26	Verification Date	: 20/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 60 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Ascites with thickened omental band with nodules with history of left breast simple mastectomy at 10/2016.
Nature of Specimen:
- Referred blocks & slide (Core biopsy & Aspiration).
- Four referred blocks labelled by (1822) including one (Tru-cut) block & three cell blocks together with one referred H&E stained slide.

Microscopic Examination:
- Tru-cut biopsy:
Histopathological examination reveals tissue cores infilrated by metastatic carcinomatous deposits, of breast origin (GATA-3 +)
- Three cell blocks slides:
* Hemorrhagic smears with reactive mesothelial cells.

Diagnosis:
Omentum, Referred blocks & slide (Core biopsy & Aspiration):
- Core biopsy:
Findings are consistent with metastatic carcinomatous deposits of breast origin.
- For further IHC (GATA3,ER, PR, Her2& Ki67).

- Aspiration (Cell blocks):
Hemorrhagic cells with reactive mesothelial cells.

Signature
Dr. Walaa Ibrahem Ghoneim.

Verified by :
Dr Maha Guimie

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL

Patient Name	: الأستاذه / هدى جمال الدين فؤاد	Request No.	: 13006052
Patient No.	: 26602052102128	Request Date	: 22/04/2026
Path No.	: SR-496/26	Verification Date	: 20/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 60 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Ascites with thickened omental band with nodules with history of left breast simple mastectomy at 10/2016.
Nature of Specimen: Referred blocks & slide (Core biopsy & Aspiration).

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) &GATA-3 clone (L50-823) .

Results: (Done on core biopsy block).
* IPT (All controls showed appropriate reactivity):
- GATA-3: Diffuse strong positive in tumor cells.
- ER: Strong nuclear reaction in 90% of tumor cells (8/8).
- PR: Strong nuclear reaction in 95% of tumor cells (8/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67: 20%.

Conclusion:
Omentum, Referred blocks & slide (Tru-cut biopsy):
Findings are consistent with metastatic carcinoma of breast origin.
IPT:
- GATA3: Diffuse strong positive.
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: 20%.

* Scoring system: "ER, PR"

Verified by :
Dr Maha Guimie

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / هدى جمال الدين فؤاد	Request No.	13006052
Patient No.	26602052102128	Request Date	22/04/2026
Path No.	SR-496/26	Verification Date	20/05/2026
Sex	Female	Financial Category	جوستاف روسي - نفقة الدولة
Age	60 Y	Department	Out Patient
Referreing Doc.			
		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim.

Verified by :
Dr Maha Guimie

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Immunohistochemistry
[[DOCTYPE:IHC]]

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Name: Hoda Gamal Al Din Fouad Ahmed *ID: 1152069*
Referred by: Prof. Ibrahim Iskander *Date: 04/04/2026*

^{18}F-FDG PET/CT examination

Clinical Indication:

- *Follow-up for patient with history of metastatic left breast cancer received interval chemotherapy, for assessment of response.*

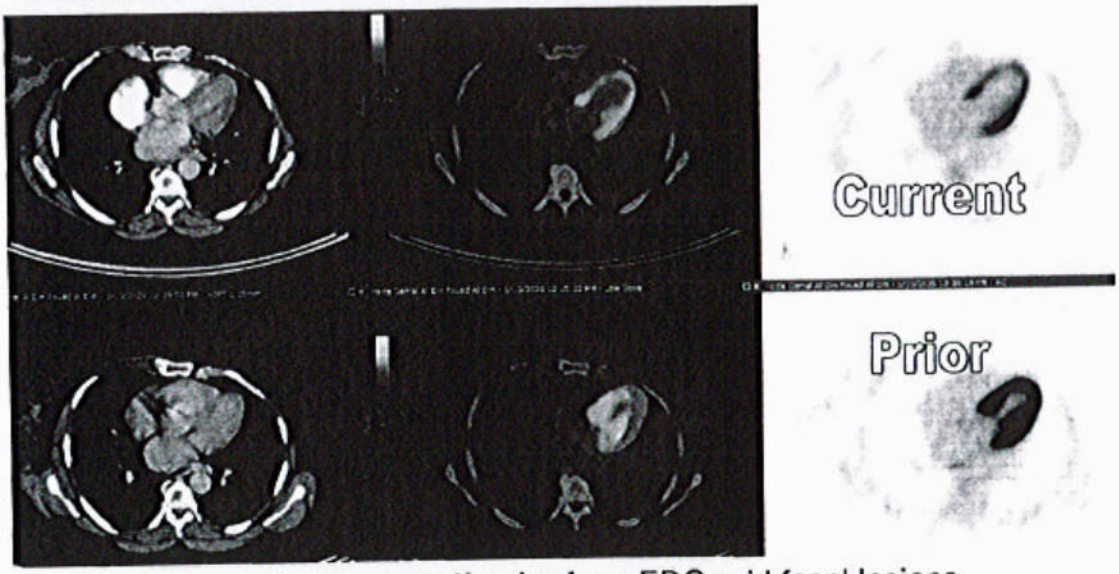
Administered activity: 299.7 MBq. ^{18}F-FDG **B.G.L.:** 184 mg/dL

Examination protocol:

- **A Whole body PET/CT study was obtained from skull vertex to the mid thigh using a low dose non contrast CT for attenuation correction**
- **Diagnostic multislice CT examination after IV contrast administration of the same region.**

Comparison was made to previous FDG PET/CT study dated 15/01/2026.

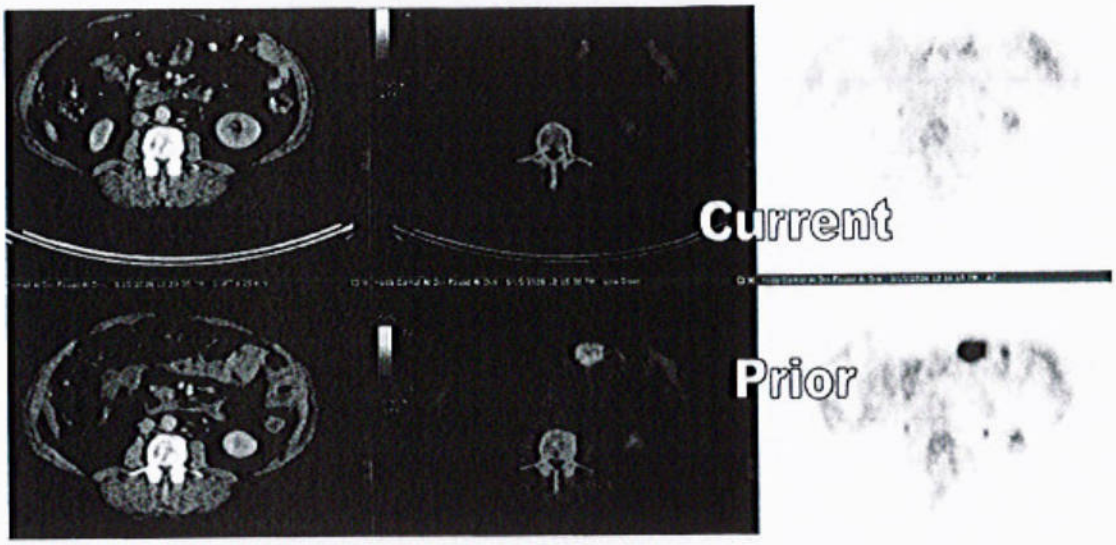
I. ***PET/CT Findings:***



- **Left mastectomy operative bed,** no FDG avid focal lesions.
- **Right breast,** no FDG avid lesions
- **Lymph nodes:**
  - **Left axillary dissection bed:** no FDG avid lesions.

PET Scan
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- No significant FDG uptake within few right axillary nodes with preserved fatty hilum.
- **No FDG avid or pathologically enlarged lymph nodes.**

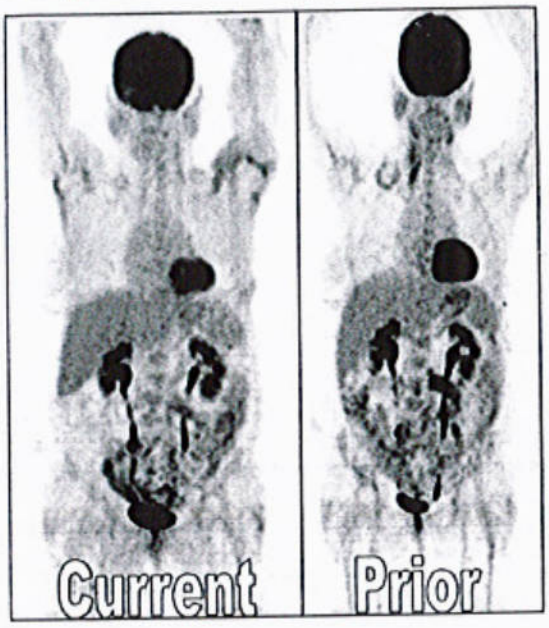


- **Peritoneum:**
  - ***Metabolic and morphologic regression of the previously noted*** nodular peritoneal thickening seen underneath abdominal wall and along pelvic reflections. SUVmax 1 compared to 1.9 as measured within lesion underneath right abdominal wall.
  - **Resolution** of the previously described mild ascites.
- **Lungs:**
  - No FDG avid pulmonary nodules.
  - **Still noted:**
    - Tiny calcified nodule in the lingula.
    - Atelectatic bands in the posterior right lung.
- **Liver:** no FDG avid focal lesions. No dilated intrahepatic biliary radicles. Patent portal vein.

PET Scan
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- **Regression of the previously described** diffuse FDG uptake **along stomach and colon.**
- **Bone marrow:**
  - No FDG avid focal lytic or sclerotic lesions.
  - **Still noted** degenerative changes of the spine.
  - **Still noted** FDG uptake related to:
    - Shoulders, arthritic.
    - Bilateral trochanteric bursitis.
- **Brain,** no FDG avid focal lesions.
- Mild regression of the FDG activity related to the right port-catheter.
- Still noted low grade FDG uptake within bilateral gluteal subcutaneous soft tissue thickening with calcifications.



II. Diagnostic CT Findings:

- No pleural or pericardial collections.
- The spleen, pancreas, adrenals, kidneys, pelvic organs, aorta and IVC appear unremarkable.
- Retro-aortic left renal vein.

OPINION:

Follow-up study demonstrates:

- **Metabolic and morphologic regression of the nodular peritoneal lesions.**
- **No newly developed hypermetabolic lesions.**

Radiologist
Dr. Mohamed Shahin, MD

Nuclear Medicine Physicians
Dr. Sherif El-Refaei, MD, EBNM
Dr. Maha Khalil, MD

PET Scan
[[DOCTYPE:PET_SCAN]]